

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°132/MVE-BDBV/23/09/2026

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HIGHLIGHTS (last 24 hours)

As of September 23, 2026, 70 new confirmed cases of Ebola Virus Disease (EVD), including 11 community deaths, were reported. These cases are distributed across the provinces of Ituri (31 cases), North Kivu (31 cases),

from Haut-Uélé (6 cases), Bas-Uélé (1 case) and Tshopo (1 case). Nine (9) deaths were recorded in the Ebola Treatment Centers (ETCs).

In total, 453 new samples were analyzed, 70 of which were found to be positive, representing a positivity rate of 15.5%.

No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CAS 7 890	ACCUMULATIVE DEATH CONFIRMED 3 799	LETHALITY 48,1 %	PATIENTS IN ISOLATION/CTE 893	ACCUMULATIVE HEALED 1 966	TRACKING RATES CONTACTS 83,4 % (Of the Day)
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Affected Provinces and Affected Health Zones



7 Provinces affected

Uele, Ituri, North Kivu,
South Kivu, Tshopo, Lower Uele and
Sud Ubangi



63 out of 167
health zones affected

(37,7%)

Summary of key points from the last 24 hours

Seventy (70) new confirmed cases of Ebola Virus Disease (EVD), including **11 community deaths**, were recorded in the last 24 hours. The provinces of Ituri and North Kivu each reported 31 confirmed cases, Haut-Uélé 6 cases, Bas-Uélé 1 case, and Tshopo 1 case.

During the same day, 9 deaths were recorded in the CTEs (6 in Ituri and 3 in North Kivu), bringing the total number of confirmed deaths for the day to 20.

On the other hand, 15 patients (**7 in Ituri, 5 in North Kivu and 3 in Haut-Uélé**) were declared cured after obtaining two negative results for Ebola virus disease.

Since the start of the epidemic in the Democratic Republic of Congo (DRC), **7 890 confirmed cases, including 3,799 deaths**, have been reported, representing a case fatality rate of **48.1 %**. The Ituri province remains the epicenter, accounting for **76.5%** of reported confirmed cases and also representing **44.3 %** of new confirmed cases.

No new health zones have been affected in the last 24 hours.

hours. Thus, **63 out of 167 health zones (37.7%)** have been affected in **7 provinces** since the start of the epidemic. Ituri has 28 out of 36 affected health zones, followed by North Kivu (16/34), Haut-Uélé (7/13), Tshopo (7/23), Bas-Uélé (3/11), South Kivu (1/34) and South Ubangi (1/16).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 23, 2026.

Province	New cases confirmed (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	31	6032	2 764	45,8%	28/36 (77,8 %)
North Kivu	31	1480	884	59,7%	16/34 (47,1 %)
Haut-Uélé	6	322	130	40,4%	7/13 (53,8 %)
Boss	1	43	15	34,9%	7/23 (30,4 %)
South Kivu	0	3	1	33,3%	1/34 (2,9 %)
Bas-Uélé	1	8	4	50,0%	3/11 (27,3%)
Sud Ubangi	0	2	1	50,0%	1/16 (6,3%)
Total	70	7 890	3 799	48,1%	63/167 (37,7 %)

Table 2. Distribution of confirmed cases and deaths by province and zone as of September 23, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	6032	2764	45,8%	31	7	6	13
Give	11	1	9,1%				0
Underland	8	6	75,0%				0
Ariwara	8	2	25,0%				0
Aungba	12	5	41,7%				0
Bamboo	197	42	21,3%	0	0	1	1
have	2	2	100,0%				0
Bunia	1683	529	31,4%	5	1	0	1
Ladies	40	9	22,5%				0
Drodro	15	7	46,7%				0
Fireworks	81	30	37,0%				0
Gethy	7	3	42,9%				0
Flounder	2	0	0,0%				0
Kilo	34	12	35,3%				0
Team	159	94	59,1%	6	1	0	1
Liter	277	139	50,2%	1	1	0	1
Logo	13	5	38,5%				0
Fight	32	8	25,0%				0
distributed	4	1	25,0%				0
Washing	24	9	37,5%	1	0		0
Darkness	75	36	48,0%	9	0	0	0
Mangala	341	153	44,9%	0	0	4	4
Mongbwalu	694	324	46,7%	2	1	0	1
Nanny	274	152	55,5%	5	3	0	3
Nizi	762	276	36,2%				0
Legumes	127	37	29,1%				0
Jungle	12	4	33,3%				0

Rwampara	1062	360	33.9%	1	0	1	1
Tchomia	76	25	32.9%	1	0	0	0
A valves	THAT	493	THAT				0
North Kivu	1480	884	59.7%	31	4	3	7
Me	306	206	67.3%	14	1	1	2
Farm	18	7	38.9%				0
Butembo	254	163	64.2%	1	0	1	1
Ten	1	0	0.0%				0
Kalunguta	50	16	32.0%	1	0		0
Cut	550	349	63.5%	2	0	1	1
Source	1	0	0.0%				0
Kyondo	33	20	60.6%	1	0		0
Lubero	21	11	52.4%	1	1		1
Be anxious.	25	11	44.0%	1	0		0
Manguredjipa	13	4	30.8%				0
Masereka	28	9	32.1%	3	0		0
The museums	121	65	53.7%				0
Mutwanga	11	2	18.2%	1	0		0
Oicha	19	13	68.4%	4	2		2
Vuh	29	8	27.6%	2	0		0
Haut-Uélé	322	130	40.4%	6	0	0	0
Kill Mangbetu	39	15	38.5%	3	0		0
Dumb	1	1	100.0%				0
Gombari	3	1	33.3%				0
Mathematics	100	43	43.0%	1	0		0
Lining	71	33	46.5%	1	0		0
Beans	2	1	50.0%				0
Ordinary	106	36	34.0%	1	0		0
Boss	43	15	34.9%	1	0	0	0
Bafwasende	7	1	14.3%				0
Kabondo	5	1	20.0%				0
Lubunga	1	0	0.0%				0
Maquis-Kisangani	23	9	39.1%	1	0		0
Mangobo	5	3	60.0%				0
Boss	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	8	4	50.0%	1	0	0	0
Blind	1	1	100.0%				0
Vladana	4	1	25.0%	1	0	0	0
Walk	3	2	66.7%				0
Sud Ubangi	2	1	50.0%	0	0	0	0
Hair	2	1	50.0%				0
Total	7890	3799	48.1%	70	11	9	20

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA : Not Applicable.

Table 3. Status of alerts notified by province for September 23, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CTE/CT (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	14	2	16	10	2	4	0	12	10
Bas-Uélé	75	3	78	14	2	61	1	16	0
Ituri	1 031	96	1127	93	83	923	0	165	82
North Kivu	810	43	853	96	43	712	0	139	72
Sud Ubangi	18	0	18	3	0	15	0	3	0
Boss	81	0	81	7	0	64	0	7	1
Total General	2 029	144	2 173	223	130	1 779	1	342	165

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

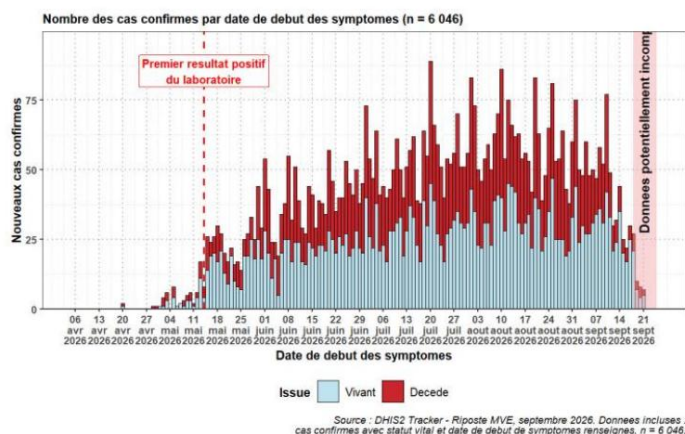
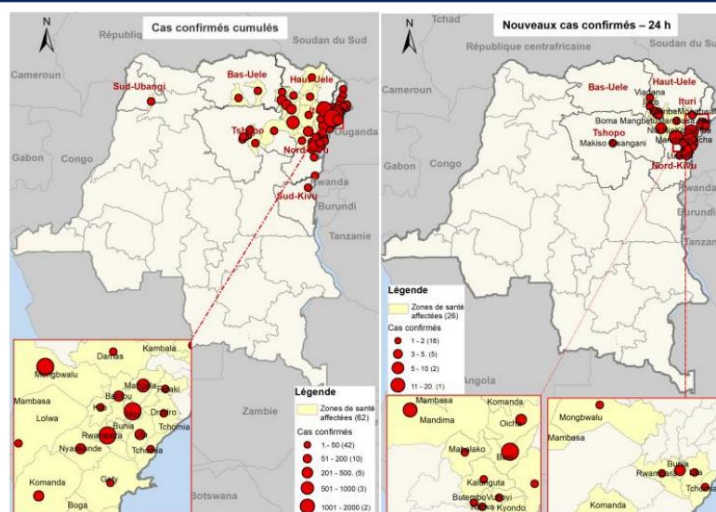


Figure 2. Spatial distribution of confirmed cases (24 hours and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Holding coordination meetings and providing support to the affected provinces.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

Based on data reported in 6 provinces, **2,173 alerts** were raised, and **2,133 (98.2%)** were verified. Following these verifications, **353 alerts** were validated as suspected cases. Of these, **342 (96.9%)** were investigated, including **165** were transferred to CT/CTE (Table 3).

Of the **32,342** contacts currently being monitored, **26,980** were seen in the last 24 hours, representing a follow-up rate of **83.4%**, higher than the previous day's rate of 78.3%. This rate was 90.7% (88/97) in South Ubangi, 88.3% (446/505) in Tshopo, 88.2% (120/136) in Bas-Uélé, 85.1% (13,615/16,005) in Ituri, 81.8% (11,090/13,564) in North Kivu, and 79.7% (1,621/2,035) in Haut-Uélé.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

One (1) alert was notified to the PoE/PoC at the PoE of Kasenyi (Tchomia Health Zone), in Ituri: it concerns a lifeless body intercepted coming from Kampala (Uganda) and destined for the morgue of Nyakasanza in Bunia; the alert was validated and burial carried out, but the body was not swabé.

Table 4. Status of key PoE/PoC indicators by province, as of September 23, 2026

Indicators	Ituri	North Kivu	Boss	Global
Report completeness	88,3% (53/60)	100% (20/20)	44,4% (8/18)	82,7% (81/98)
% of functional strategic PoE/PoC	100%	100%	100%	100%
Number of people who have passed PoE/PoC	127 161	97 094	19 541	243 796
% of travelers screened	98,4%	97,6%	98,9%	98,1%
% of travelers who washed their hands	98,0%	97,6%	99,2%	98,0%
% of travelers made aware	99,4%	97,6%	100,0%	98,8%
Number of alerts notified	1	0	0	1
Number of validated live alerts (cas suspects)	0	0	0	0
Number of bodies intercepted today % of suspected	1	0	0	1
cases transferred to CTE/CT	THAT	THAT	THAT	THAT
% of lifeless bodies swabed/secured	0,0% (0/1)	THAT	THAT	0,0% (0/1)
% of suspected cases investigated within 4 h	THAT	THAT	THAT	THAT

2.2.2. Challenges

Lifeless body intercepted at Kasenyi PoE not swabé; in Ituri, six PoCs in the Nia-Nia health zone not functional due to lack of payment, Avakubi PoC closed due to insecurity, low encoding of alerts in DHIS2 Tracker and insufficient mobility in the Rwampara, Nizi and Bunia health zones; in North Kivu, persistent threats against PoC providers on the Butembo axis; in Tshopo, low completeness of reports (44.4%; 8/18); absence of PoE/PoC report from Haut-Uélé.

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 31 new positive results** (24 living and 7 deaths) out of 257 new samples received and analyzed (**positivity: 12.1%**).
- **North Kivu: 31 new positive results** (27 living and 4 dead) out of 162 samples received and analyzed (**positivity: 19.1%**).
- **Haut-Uélé: 6 new positive results** (6 living and 0 deaths) out of 26 samples received and analyzed (**positivity: 23.1%**).
- **Tshopo: 1 new positive result (1 live sample)** out of 4 samples received and analyzed (**positivity: 25.0%**).
- **Bas Uele: 1 new positive result (1 live sample)** out of 4 samples received and analyzed (**positivity: 25.0%**).

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **In Ituri**, 15 out of 30 rings were opened (50.0%) and 50 out of 52 identified households were decontaminated (96.2%); all 8 of the 8 FOSA concerned were decontaminated, as well as 25

In schools, 30 households received hygiene kits in Mongbwalu and 14 health and social care establishments (ESS) received infection prevention and control (IPC) kits in Bunia. Regarding health and safety reports (HSRs), 91 alerts were recorded (including 9 postponements from the previous day), 55 HSRs were carried out (60.4%), and 66 bodies were swabbed; 25 failed swabbings were reported.

- **In North Kivu**, 36 out of 46 infection control rings were opened (78.3%), 15 out of 42 households were decontaminated (35.7%), and 40 out of 45 health facilities were decontaminated (88.9%). Six infection prevention and control (IPC) scorecard assessments were conducted in six health establishments (HES) with an average score of 30.4%. Staff exposure risk assessments classified 7 out of 22 staff members as high risk in Beni, and IPC training for hygiene workers was completed in Beni, Mabalako, Mutwanga, Kalunguta, Butembo, and Katwa with support from IMA/MOMENTUM. The timeliness and completeness of reports reached 75.0% (12/16 health zones). Regarding blood and health surveillance (BHS), 55 alerts were received (plus 22 postponements from the previous day), 55 bodies were successfully swabbed, and 60 BHS were conducted; 17 alerts were postponed to the following day.
- **In Tshopo**, decontamination of the household of a confirmed case in the Kabondo health zone (3 rooms, a latrine, a shower, and furniture, with incineration of a mattress); 15 community health workers (CHWs) were monitored and supported in 6 health zones, with 164 service providers briefed on standard precautions, 500 liters of water provided in public places in Wanie-Rukula, and validation of the CHW daily infection prevention and control (IPC) reporting templates for CHWs. Regarding the emergency response teams (ERDs), an alert was received (a body intercepted at the Point of Control (POC) PK 23 on National Route 3).
- **In Bas-Uélé**, seven health centers (ESS) in the health zones of Buta, Ganga, and Viadana were monitored in the implementation of their improvement plans. 36 out of 40 service providers (90.0%) and 11 out of 13 hygiene workers (84.6%) were briefed. 628 students and teachers at the Viadana Primary School were made aware of barrier measures, and the isolation room at the Buta General Referral Hospital was decontaminated. No contact tracing has yet been initiated around the new case in Viadana. Regarding the health surveillance systems (EDS), three alerts were received (two in Ganga and one in Koteli), two bodies were swabbed, and no EDS tests were conducted.

2.4.2. Challenges

In Ituri: Ring road openings limited to 50.0% (15/30); 25 swab failures and 11 EDS alerts postponed; resistance in Bambu, Nizi, Rwampara, and Aru; and insufficient transportation in Kilo, Nia-Nia, Mandima, and Fataki. In North Kivu: Low household decontamination rates (35.7%; 15/42); no IPC kits distributed to households of cases (0/15); lack of traceability for some eligible households; and the need to activate and train decontamination teams in Biena, Masereka, Kyondo, Mabalako, and Vuhovi.

2.5. Holistic care

2.5.1. Main actions

- **In Ituri**, 63 new admissions were recorded during the day, compared to 46 discharges, including 7 recoveries. Thus, 440 patients are hospitalized out of 1015 beds (occupancy rate of 43.3%); 406 out of 425 patients (95.5%) received hot meals, and children under LNPE/SLM were fully covered at the CTE (14/14), at the nursery (3/3), and in the community (27/27); 34 patients are in critical condition.
- **In North Kivu**, 70 new admissions were recorded, compared to 67 discharges, including 5 recoveries; 369 patients are hospitalized, 250 of whom are in standardized care facilities, representing an occupancy rate of 81.2%; 119 patients (32.2%) remain under the care of specialized healthcare facilities (ESS) or treatment centers (CTE), and 23 patients are in critical condition. All 168 patients, 98 accompanying persons, and 328 frontline providers received hot meals (100%).
- **In Haut-Uélé**, 12 new admissions were recorded against 3 discharges (3 recovered). Thus, 62 patients are hospitalized for 138 beds, representing an occupancy rate of 44.9%.
- **In Tshopo**, no new admissions or discharges were recorded. Sixteen (16) patients (8 confirmed, including 6 PPL, and 8 suspected) are in isolation for 31 beds, representing an occupancy rate of 51.6%; 8 confirmed, 5 suspected, and 3 accompanying persons received a hot meal, and 42/42 (100%) frontline providers also received hot meals.
- **In Bas-Uélé**, 2 new admissions of suspected cases, compared to 4 discharges of non-cases; 6 patients (1 confirmed in Ganga and 5 suspected cases) remain in isolation in non-standardized CTs of

Ganga, Poko, Limba and Buta, while the new confirmed case from Viadana has been transferred to the CTE in Isiro.

2.5.2. Challenges

Treatment remains partially substandard and capacity is limited: **In Ituri**, Ebola Treatment Centers (ETCs) are saturated with suspected cases in Nizi, Mambasa, Lolwa, Kilo, Mangala, and Mandima. **In North Kivu**, 32.2% of hospitalized patients are treated outside of standardized facilities; no beds are available in Butembo and Katwa. **In Tshopo**, the lack of safe blood and limited technical resources persist. **In Bas-Uélé**, the absence of ETCs and ambulances hinders the isolation of cases.

2.6. Vaccination

2.6.1. Main actions

- **In Tshopo**, 4,058 TPL and PPL were vaccinated out of 3,549 pre-registered (114 %), representing 35% of the target of 11,703, including 2,035 in Makiso-Kisangani, 521 in Kabondo, 458 in Bafwasende and 456 in Mangobo; 4,300 doses were distributed to the health zones, including 300 on September 22, and 442 thawed doses remain in stock at the hub depot, expiring on October 5, 2026. In Ituri, 66 PPL were vaccinated in Bunia from September 21 to 23, bringing the cumulative total to 96 out of a target of 300 TPL and PPL (32%).

2.6.2. Challenges

Low adherence of PPL and TPL to vaccination in Tshopo (35% of the target), where coverage remains low in Tshopo (38% of pre-registered) and in Lubunga (61%), and data from two sites in Bafwasende are pending; in Ituri, coverage of PPL is still limited in Bunia (32% of the target); in Bas-Uélé, absence of a micro-plan for PPL vaccination in the 6 priority health zones and need for additional doses of ERVEBO.

2.7. Continuity of Care

2.7.1. Main actions • In Tshopo, 29

confirmed cases under experimental treatment (3 under MBP134 and 26 under remdesivir: 10 cured, 7 deceased, 1 escaped and 8 in treatment) and 185 people have benefited from post-exposure prophylaxis (101 completed, 12 interrupted and 72 in progress); the evaluation of the Kawaki clinic revealed an ineffective triage area and less than 5% of staff vaccinated (1/21), and the temporary closure of the ESS DHKis is recommended in the face of the multiplication of confirmed and suspected cases.

- **In Ituri**, the development of the recovery plan for the CSSE of the HGR in Bunia and the survey on the impact of Ebola on the use of reproductive health services, supported by MDA and Africa CDC, are continuing.
- **In Haut-Uélé**, the initial assessment of the HGR shows overall continuity ensured in Dungu. Gombari and Rungu; followed by 5 PPLs assigned (3 to Wamba and 2 to Pawa).

2.7.2. Challenges

Major drug shortages in Boma Mangbetu, Isiro, Wamba and Pawa.

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In Ituri**, 14,191 home visits reached 65,748 people and 829 educational talks benefited 16,228 people; 108 mass awareness campaigns reached 6,183 people, 17 community debates (788 participants) and 9 advocacy sessions with 26 people; 259 community alerts were reported, including 35 deaths, 153 feedback requests were received, of which 87 were resolved (56.9%), and 31 community incidents were reported, of which 11 were resolved (35.5%), for a completeness rate of 35.7% (10/28 health zones); 56 public service announcements were broadcast on 15 radio stations and 6 programs were hosted, and 72 people, including 12 school principals and 30 teachers, were sensitized in 13 schools in the Mambasa sub-division
- **In North Kivu**, 10,161 home visits reached 32,465 people, 87 educational talks brought together 1,372 people, 16 mass communication sessions reached 5,231 people, and 10 briefings reached 437 people; 214 alerts of suspected cases and deaths were notified by the

community actors out of 450 expected (47.6%), 18 out of 23 feedbacks were resolved within 48 hours (78.3%), 6 out of 9 incidents were resolved (66.7%) and 1,367 out of 4,750 people were engaged in community actions (28.8%); a public forum was organized in the Wayene AS (Katwa ZS) to overcome resistance.

- **In Haut-Uélé**, 550 home visits reached 2,441 people, 69 educational talks (591 people reached) and 10 mass activities reached 905 people in Isiro and Wamba; 49 ECZ members and community actors were trained on CREC key indicators and community-based monitoring, the pygmies of Malikuti village were sensitized, an interactive program was hosted on RTK and 36 feedbacks were resolved within 48 hours.
- **In Tshopo**, 264 home visits reached 1,480 people, 3 educational talks (reaching 182 people) and 6 mass awareness campaigns (reaching 1,480 people); 12 provincial deputies were briefed at the Provincial Assembly, followed by a visit from the Meteorological Technical Center (CTE Météo), and their commitment to supporting the response was obtained; 50 community alerts were notified and all addressed, 2 feedback requests and 2 refusals resolved, 389 posters and 235 leaflets distributed, and 5 radio programs broadcast.
- **In Bas-Uélé**, 220 home visits reached 789 people, 5 educational talks reached 18 people and 3 mass awareness campaigns reached 2,311 people in the health zones of Ganga, Poko and Buta; 9 community alerts were notified and all dealt with, advocacy was carried out with local authorities for the non-stigmatizing reintegration of the first person who recovered from the Ganga CTE and the CREC supported the swab of two lifeless bodies in the Ganga health zone.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- **In Ituri**, 14 of the 35 confirmed cases (40.0%) from the previous day and 107 of the 190 confirmed cases under follow-up (56.3%) benefited from SMSPS interventions, as well as 28 of the 61 new suspects (45.9%) and 116 of the 176 suspects under follow-up (65.9%); 189 caregivers and 43 PPLs were supported, the 24 recovered were followed up in the community (100%) and 53 results were announced, including 14 positive results; 11 of the 28 health zones are covered, 11 children are being followed up in daycare and 14 of the 16 households of separated and orphaned children were supported.
- **In North Kivu**, 10 of the last 11 confirmed cases (90.9%) and 219 confirmed cases under follow-up benefited from SMSPS interventions, as well as 219 new suspected cases and 346 suspected cases under follow-up; 76 households were supported for public health actions and 89 PPLs were supported, 7 recovered out of 14 followed up in the community (50.0%), 20 of the 24 households of separated and orphaned children were supported and 96 results were announced, including 13 positive results; 5 of the 6 health zones that notified are covered and 176 men, 218 women and 79 children benefited from psychoeducation sessions with the support of UNICEF.
- **In Tshopo**, the 8 confirmed cases under follow-up at the CTE Météo (100%) and the 12 suspected cases under follow-up (100%) benefited from SMSPS interventions; 201 people benefited from psychoeducation, including 127 students from the Tomibikisa Institute and 16 student interns in Kabondo, 25 accompanying persons at the CTE Météo and 9 PPL, one household was prepared for public health actions and the resistance notified in Kabondo was broken (1/1).

2.9.2. Challenges

- **In Ituri**, low coverage of SMSPS interventions (11 of the 28 health zones covered) and a need to strengthen collaboration between pillars for joint interventions around the latest confirmed cases; **in North Kivu**, a lack of nurseries in 10 affected health zones and insufficient human resources and logistical means for the pillar; **in Tshopo**, need to train the APS of the ZS of Wanie-Rukula and Bafwasende and to support the pillar in communication units and means of transport.

2.10. Logistics

2.10.1. Main actions

- Support for the mobility of response teams in all affected provinces; in North Kivu, handover of 10 motorcycles to the Beni Health Zone, continuation of the installation of equipment in the Presidential Task Force coordination building and construction work on the Ebola Treatment Center at Musienene General Referral Hospital; in Tshopo, reception of laboratory supplies from Kinshasa and Bunia (INSP) and IPC supplies (Enabel), and monitoring of the construction of the Bafwasende ETC; in Bas-Uélé, reception of laboratory supplies and equipment, construction of the Buta General Referral Hospital transit center and rehabilitation of the DPS offices with the support of WHO; corrective maintenance of vehicles and support for the deployment of IPC supplies, care, communication, PoE/PoC and vaccination.

2.10.2. Security

2.10.3. Main actions

- Continued security monitoring in the affected provinces; in North Kivu, the EDS and PCI teams were accompanied in the sub-coordinations of Beni and Katwa (100%) for the burial of 4 confirmed cases, the decontamination of 2 households and 3 health structures (CH Kasanga, CM La Fraternité and CM La Bénédicte) as well as the swab and securing of bodies in Kyaghala, Vighole and the CTE of Katwa.

2.11. PSEA (Prevention of Sexual Exploitation and Abuse)

2.11.1. Main actions

- In Ituri, 25 CTE Lita executives and UNDP actors (16 men and 9 women) were briefed on the PSEAH, of which 14 signed the code of good conduct, and 97 people were sensitized in the ZS of Bunia and Gethy (traders of the Fichama and Kinida markets, motorcyclists and teachers); the display of messages and the distribution of leaflets continue in Bunia, Aru, Mongbwalu, Mambasa and Fataki.
- At the national coordination level, 13 of the 36 health zones (36.1%) are aware of the reporting mechanism and two partners (WHO and Save the Children) have reported their PSEA activities.

2.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 41 of the 119 health zones monitored (34.5%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of alignment by some partners; insufficient mass awareness materials and lack of means of transport and computer kits for the pillar.

MESSAGES CLES

- Active and persistent community transmission in the provinces of Ituri, North Kivu, Haut-Uélé, Tshopo and Bas-Uélé. However, the provinces of Sud-Ubangi and Bas-Uélé report sporadically. Progress to be consolidated is being made in the area of
Epidemiological investigations: almost all (96.9 %) of the suspects were investigated within 24 hours of the suspected cases being investigated, although the rate of contact tracing fluctuates (83.4%).
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to rapidly interrupt transmission chains: Rapidly increase reception capacities in ETC/CT in North Kivu and strengthen cross-border surveillance with South Sudan.
- Strengthen preparedness and response to Ebola Virus Disease-B in the provinces of the River Corridor and surrounding areas.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



TASK FORCE PRESIDENTIAL EBOLA 17
Ministry of Public Health, Hygiene and Social Welfare
National Institute of Public Health
Public Health Emergency Operations Center
MVE17 Incident Management System

Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Email: dieudonne.mwamba@insp.cd

Incident Manager

Professor Dr. AHUKA MUNDEKE Steve

Email: steve.ahuka@unikin.ac.cd

Operations Manager

Dr. KOBBE NGWAMA John

Editorial Team

Professor Dr. MULANGU KATULUMBA Félix-Médard

Dr. KATUSI BISELENGE Jules

Mrs. WISE DEAD Marie

Dr. DIUR MANANASHI Floryan

Mr. Kamuina Kebela John

Contact :

www.insp.cd

contact@sgiebola17.org

Freephone number: 151. www.sante.gouv.cd